

Singapore–Cambridge GCE O Level English Language (Syllabus 1184) Paper 2 — Comprehension Practice Paper 8

Time allowed: 1 hour 50 minutes

Total marks for Sections B and C: 45

Instructions to candidates

- Answer **all** questions in Sections B and C.
- The number of marks is given in brackets [] at the end of each question.
- Answer in **complete sentences** where instructed.
- For the summary in Section C, write **no more than 80 words**, not counting the words given to help you begin.

Section B — Text 3 (Narrative) [20 marks]

Text 3: A retired pilot takes one last flight in a small Cessna over the coast where he learnt to fly fifty years ago.

1

The hangar smelt of old oil and warm aluminium, exactly as it had in the summer of 1974. I rested a palm on the Cessna's cowl and was surprised, foolishly, to find it cool — I had half expected the little aeroplane to greet me with the same affectionate warmth a dog reserves for an old master. She was forty-eight years old, almost the same age as my younger daughter, and she had outlasted three of my marriages and most of my friends. I walked around her slowly, as one is taught to do, running my fingers along the leading edge of each wing, checking the hinges, the tyres, the trim tabs. The ritual has not changed. I have.

2

My instructor, fifty years ago on this same strip of coast, had drilled the pre-flight check into me until I could perform it half asleep, which on certain Monday mornings I more or less did. "If you skip the walkaround," he used to say, "the sky will find what you missed." He had died of a perfectly ordinary heart attack in 1989, in his garden, and I had thought of him perhaps once a week ever since. Today I thought of him as I tugged each control surface — elevator, ailerons, rudder — and listened for the small, clean click that meant the cable was free. Everything answered. The aeroplane was ready before I was.

3

I had told no one this was the last flight. My daughters had been kind, in that careful adult way, about my decision to surrender the medical certificate at the end of the month; they had not asked whether I planned a farewell. I had not planned one either, exactly. I had simply driven down before dawn, signed the booking sheet in my own slightly trembling hand, and pushed open the hangar doors to a sky the colour of a cracked oyster shell. Whatever this was, it would happen without an audience.

4

The engine caught on the second turn of the propeller, as it always had. I taxied to the threshold, ran through the run-up checks, and called the empty tower out of habit. A surprised young voice answered, cleared me for departure, and wished me a good morning. I rolled onto the runway. There is a moment, in a light aircraft, when the wheels lose interest in the ground — they do not so much leave it as forget about it — and that moment, after fifty years, still ambushed me. My eyes filled, briefly and without permission. I corrected the aircraft, blinked twice, and climbed.

5

The coast unrolled below me as it had unrolled then: the white scribble of surf, the patchwork of green fields tipped slightly towards the sea, the lighthouse on the headland still keeping its lonely appointments. I followed the old training route in reverse — north along the cliffs, a gentle turn over the estuary, then west towards the line of hills where, as a student, I had been asked to make my first solo landing on a strip of bumpy grass that no longer existed. I throttled back and let the Cessna drift. Somewhere below, a younger man was learning what it meant to be alone in the air. I let him learn.

6

At three thousand feet, somewhere between the estuary and the hills, the engine coughed. It is a small word, **coughed**, and quite the wrong one for what it does to a pilot's heart. The needle on the fuel pressure gauge twitched downwards, hesitated, and steadied again. I switched tanks, eased the mixture, and watched. The engine cleared its throat and resumed its proper purr, but for perhaps thirty seconds I had been very alert, very alive, and quite certain that the sky was about to find what I had missed. My instructor would have approved. I laughed, just once, into my oxygen mask. It was not a comfortable laugh.

7

I turned for home. The hills slid back into the haze. The estuary widened beneath me, then narrowed again into the familiar runway, with its tired windsock and its single peeling hangar. I flew the circuit by the numbers — downwind, base, final — as carefully as a student on check ride. The wheels found the tarmac with the gentle bump of an old friend remembering a handshake. I taxied off, shut down the engine, and sat for a long minute in the sudden, ringing silence. Then I climbed out, patted the cowling once, and walked across the apron to where my car waited. I did not look back. I had been taught, long ago, that a pilot who keeps glancing over his shoulder is no longer flying the aircraft in front of him.

From Paragraph 1

1. The narrator says he was “surprised, foolishly” to find the aeroplane cool to the touch. Explain, in your own words, what this reaction suggests about how he feels towards the aircraft. [2]

2. The narrator says, “The ritual has not changed. I have.” What is the **effect** of using these two short sentences at the end of Paragraph 1? [2]

From Paragraph 2

3. The instructor used to say, “If you skip the walkaround, the sky will find what you missed.” Explain, in your own words, what the instructor meant by this. [2]

From Paragraph 3

4. Give **two** details from Paragraph 3 that suggest the narrator wanted his final flight to be a **private** occasion. [2]

From Paragraph 4

5. The narrator says that the wheels “do not so much leave it as forget about it.”
- (a) Explain, in your own words, what this description suggests about the moment of take-off. [1]
- (b) Why is this moment said to “still ambush” the narrator? [1]

From Paragraph 5

6. The narrator says the lighthouse was “still keeping its lonely appointments.” Explain how the language used here makes the lighthouse seem more than just a building. [2]

7. The narrator says, “Somewhere below, a younger man was learning what it meant to be alone in the air. I let him learn.” What does this tell us about how the narrator now sees his younger self? [2]

From Paragraph 6

8. The narrator describes the engine’s hesitation by saying it “coughed.” Explain what is **effective** about choosing this word. [2]

9. Why does the narrator say his laugh into the oxygen mask “was not a comfortable laugh”? [1]

From Paragraph 7

10. The narrator says the wheels “found the tarmac with the gentle bump of an old friend remembering a handshake.” Explain how this comparison reflects the narrator’s feelings about the landing. [2]

11. What is the **tone** of the final sentence of the text? [1]

Section C — Text 4 (Non-narrative) [25 marks]

Text 4: A feature article on the promise and the perils of artificial intelligence in healthcare.

1

In a hospital in Seoul last year, an artificial-intelligence system flagged a small shadow on a chest scan that two human radiologists, working independently, had reviewed and dismissed. The shadow turned out to be an early-stage lung cancer. The patient, a fifty-one-year-old teacher, is now in remission. Stories of this sort — quiet, undramatic, and increasingly common — explain why hospitals around the world are pouring money into machine-learning tools that promise to read scans, sort patients, and predict deterioration with a speed and consistency that no tired human can hope to match. They also explain why a growing number of doctors are uneasy.

2

The clearest successes have come in diagnostic imaging. Algorithms trained on millions of labelled X-rays, mammograms and retinal photographs now detect certain cancers, fractures and signs of diabetic eye disease at rates that match — and, in several published trials, modestly exceed — those of experienced specialists. In emergency departments, triage systems built on similar techniques are being used to flag the patients most likely to deteriorate in the next four hours, allowing nurses to direct attention where it is most needed. Used well, these tools do not replace the clinician; they sharpen her eye and lengthen her day.

3

Yet “used well” is doing an enormous amount of work in that sentence. The performance of a medical algorithm depends, almost entirely, on the data it was trained on. A model fed mostly on chest scans of middle-aged white men will, predictably, perform less reliably on the chest scans of elderly Asian women. Recent audits of widely deployed dermatology tools have shown that they are markedly less accurate on darker skin, because darker skin was under-represented in their training images. The fault, it should be said, is not the algorithm’s. It is ours: we built it on a world that we had only half photographed.

4

There is also the more uncomfortable problem of over-reliance. A junior doctor, presented with an “AI-confirmed” diagnosis on her screen, may find it harder to challenge the machine than to challenge a senior colleague, and harder still to record her doubts in the patient’s notes. Studies of radiology departments that have introduced decision-support software report a measurable, if uneven, decline in the rate at which junior staff overrule the system — even when, on review, the human reading was the correct one. The danger here is subtle. It is not that the algorithm is wrong; it is that the doctor begins, gradually and politely, to stop thinking.

5

Who, then, is responsible when an algorithm errs? The hospital that bought the software? The clinician who acted on its advice? The company that wrote the code and trained the model on data she has never seen? At present, the law in most jurisdictions answers, somewhat awkwardly, “the doctor” — on the grounds that the clinician is the human in the loop. This may be defensible while AI is still a junior partner in the consulting room, but as the partnership matures, the question of accountability will not stay quiet. Patients, asked in a recent survey across four countries, were notably less willing to accept an AI-led decision when they could not identify a human who was, in the end, answerable for it.

6

Regulators, predictably, are scrambling. The European Union’s AI Act, the United States Food and Drug Administration’s evolving framework for “software as a medical device”, and Singapore’s Health Sciences Authority guidelines all attempt, in slightly different vocabularies, to draw a line between tools that

merely **assist** a clinician and tools that effectively **decide** on her behalf. The line is harder to draw than it sounds. A piece of software that “merely flags” a tumour for a tired radiologist at three in the morning is, in any practical sense, deciding which patient she will look at first. Regulation that ignores this is regulation that does not yet understand what it is regulating.

7

None of this is an argument against medical AI. The teacher in Seoul is alive because of it, and so, by now, are tens of thousands of others. It is, however, an argument for honest installation: for hospitals that train their staff to **use** these tools rather than merely defer to them, for companies that publish the limits of their models as clearly as they publish the strengths, and for a public conversation that treats the machine as what it actually is — a powerful, fallible apprentice working under a tired and accountable master. The promise of AI in medicine is real. So is the temptation, on every busy ward, to forget the second half of the sentence.

From Paragraph 1

12. The writer describes the success stories as “quiet, undramatic, and increasingly common.” Explain, in your own words, what this phrase suggests about how AI is changing medicine. [2]

13. According to Paragraph 1, give **two** reasons why hospitals are investing in machine-learning tools. [2]

From Paragraph 2

14. The writer says that AI tools “do not replace the clinician; they sharpen her eye and lengthen her day.” Explain, in your own words, what the writer means by this. [2]

From Paragraph 3

15. Why does the writer place the phrase **used well** in inverted commas at the start of Paragraph 3? [2]

16. The writer says, “The fault, it should be said, is not the algorithm’s. It is ours: we built it on a world that we had only half photographed.” Explain, in your own words, what the writer means by this. [2]

From Paragraph 4

17. The writer says that the danger of over-reliance is that “the doctor begins, gradually and politely, to stop thinking.” What is the **effect** of the words **gradually and politely** in this sentence? [2]

From Paragraph 5

18. The writer asks three questions in a row at the start of Paragraph 5. Why does he use this technique? [2]

19. According to Paragraph 5, why are patients uneasy about an “AI-led decision”? Answer in your own words. [1]

From Paragraph 6

20. The writer says that a piece of software that “merely flags” a tumour is “in any practical sense, deciding.” What is the writer’s **tone** in this sentence? [1]

From Paragraph 7

21. The writer describes medical AI as “a powerful, fallible apprentice working under a tired and accountable master.” Explain what is **effective** about this description. [2]

22. Summary question [6 marks for content, 7 marks for use of own words and style]

Using your own words as far as possible, summarise **the benefits** that artificial intelligence is bringing to healthcare, and **the risks** it carries, as described in the text.

Use **only** material from **Paragraphs 2 to 6** of Text 4.

Your summary must be in **continuous writing** (not note form). It must **not** be longer than **80 words** (not counting the words given to help you begin).

Begin your summary as follows:

Artificial intelligence is helping healthcare by ...

— END OF PAPER —